

APPENDIX 4 - HIGH MED CONSORTIUM MOBILISATION RISK REGISTER

No.	Area of Risk	What is the potential risk?	What is the consequence if the risk happens	Impact	Likelihood	Score (xL)	What are the Initial Plans to mitigate the risk?	Impact	Likelihood	Score (xL)	Risk Owner
1	Contract Completion	Failure to complete contracts	Delays in process, lost momentum and lack of engagement. Any change needs to be bought into by internal staff, otherwise new working arrangements won't be as effective and will be harder to embed.	3	4	12	- Partners and our Solicitor has reviewed draft contracts and we do not foresee any complications. - Maintain open communication channels with the area team. - Ensure availability of directors at contract completion. - Ensure partners maintain viability as a contractor.	3	2	6	SM/GS
2	Issue Consultation report with feedback and proposed actions, inviting further feedback	Failure to complete Patient Consultation	Unable to engage and build trusts with patients, this could lead to a resistance to any changes or improvements that we implement. Any new initiatives may not have their input or buy-in.	3	4	12	We have a Patient Engagement Plan that will engage patients early, and ensure they feel empowered to voice their opinions and recognise how much it will benefit the practice and their care. Additionally, we have undertaken significant research into patient concerns, the current availability of appointment times and patient need voiced in patient engagement report, NHS Choices, and via Healthwatch and GP Survey. Any options implemented will be made to increase positive patient experience, outcomes and appointment availability. Our well formed plans to ensure we TUPE over all staff will maintain patient rapport, upon which we can build an engaged and involved patient population.	2	2	4	MK
3	TUPE Completion	Failure to Complete TUPE	Loss of capacity, and legacy skills and knowledge	4	3	12	Comprehensive plan in place, focusing on early engagement with the incumbent provider and key TUPE staff first. Involvement of transferring staff will maximise TUPE conversion. Together our team have overseen successful transfer of all staff in our last 5 Mobilisations. Additionally, current staff can take over the senior roles until backfills secured. We have a bank of regular sessional GPs local to NWL as additional contingency.	2	1	2	FN
4	Confirm complete Workforce in place for service commencement, including implementation of management	Failure to confirm full staff model	Possible impacts on the ability and capacity to deliver quality care and meet Hounslow Standard KPIs	4	4	16	We have already got pre-existing relationships with a number of the TUPE staff and strong TUPE plans aligned to ACAS and best practice. We have already identified and agreed the additional GP time that we require for the contract - terms have been agreed subject to contract award. For other staff we have recruitment plans in place. Additional contingency will be provided by our network of local GPs, we have long-term relationships with, and for extra contingency we will engage the regular locums already working with the practice	2	3	6	MK
5	Finalise and Sign Property Lease including FM and Maintenance service agreements	Failure to complete Lease	Delay in securing long-term could lead to delays in making changes to Practice environment, and limit ability to plan	2	5	10	Our team have tenures with NHS Property Services and other landlords in our other clinics, so have a good Landlord/Tenant relationships . We are familiar with the lease agreements used, and both GP Partners/Solicitors are happy with draft terms. They are also happy to sign any interim agreements if required.	2	2	4	FN/US

6	Sign-off and Final Assurance of records transfer	Failure to agree joint merger plans	Possible issues with data integrity and delivery of care continuity	4	3	12	Our specialist Data Manager would lead data migration, with support from Dr. Fareed Nasir, he has successfully undertaken in collaboration with the NWL ICB, the reconciliation of over 20,000 records in the past. Our plan will be informed by lessons learned having effectively merged the two legacy practices of HESA Medical Centre, and subsequent dispersal/merger of former Kingsway Surgery into HESA's patient list. We will meet with the ICB during stakeholder engagement to discuss our experiences and their recommendations to develop the best collaborative strategy.	2	1	2	SN
7	Commissioner validate Business as usual	Failure to achieve BAU validation	Possible further reviews and action plans would be required to ensure that the service was validated	3	3	9	We have a comprehensive overarching Mobilisation Plan, and we will take an individual approach to meet each stakeholders needs; progressing each to pass through each stage Gate in order for the overall plan to be met. We will maintain open communication channels with NWL and other stakeholders to provide regular updates against Critical milestones. Weekly Implementation and Risk reviews will be undertaken. Reports to the commissioner regarding mobilisation will keep momentum and progress on track. Any possible delay will be identified and addressed swiftly.	2	1	2	US